



Hyperactive Delirium with Severe Agitation

Assessment

Pediatric Pearls:

- Use pediatric dosing of medications or electrical therapy for a pediatric patient < 37 kg, as defined by HandTevy.

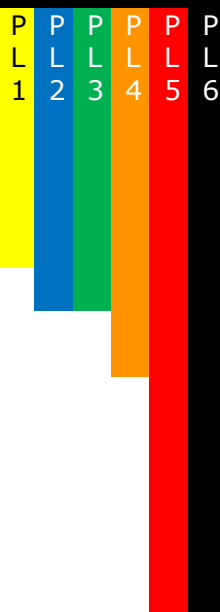
Signs & Symptoms:

- Anxiety, agitation, confusion
- Affect change, hallucinations
- Delusional thoughts, bizarre behavior
- Tachycardia, diaphoresis, tachypnea
- Struggles violently despite appropriate restraint
- Combative/violent
- Very "hot" to touch

Differential:

- Metabolic/endocrine disorders
- Head injury, encephalitis, seizure
- Medication effect/overdose, Serotonin Syndrome, polypharmacy
- Medication stoppage
- Bipolar (manic-depressive)
- Schizophrenia, anxiety disorders, etc.
- Alcohol, amphetamine, cocaine

Clinical Management Options



- [Oxygen](#) ≥6lpm titrate to 95-100% SpO₂
- [BGL Assessment](#)
- Temperature assessment
- Verbal de-escalation
- Basic Airway Management as needed
- [Physical restraint](#), if needed, and use [sedation checklist](#)
- Cooling measures if needed
- Head elevated 30°
- Continuous cardiac, ETCO₂, and SpO₂ monitoring
- [Vascular access](#) as appropriate for patient condition
- Fluid therapy as needed with [isotonic crystalloid](#), preferred cold
- [12 ECG](#)
- If the patient suffers cardiac arrest, consider a fluid bolus and [Sodium Bicarbonate](#) early
- RASS +4 (Hyperactive Delirium with Severe Agitation) - [Sedation Checklist](#)
- RASS +2 or +3 (aggressive behaviors requiring sedation) – [Behavioral COG](#)
- RASS +1 or +2 (uncontrolled anxiety) – [Behavioral COG](#)
- [Advanced Airway Management](#) as needed

Consult Online Medical Control as Needed

Modified Out-of-Hospital RASS

- +4 Violent/ Immediate danger & unable to be consistently restrained
- +3 Aggressive movement: Offensive behavior directed at people, most "pushing" type movement
- +2 Vigorous movement: Most "pulling" type movement / defensive behavior
- +1 Restless: Uncontrolled Anxiety, movements not aggressive or vigorous
- 0 Alert and Calm: Spontaneously pays attention to provider
- 1 Drowsy: Not fully alert but sustains more than 10 seconds wake, with eye opening in response to verbal command
- 2 Light sedation: Awakens briefly for less than 10 seconds with eye contact or verbal command
- 3 Moderate sedation: Any movement, except eye contact, in response to command
- 4 Deep sedation: No response to voice, but any movement to physical stimulation
- 5 Unarousable: No response to voice or physical stimulation



Hyperactive Delirium with Severe Agitation

Pearls:

- Consider your safety first. Physical restraint should be performed/assisted by Law Enforcement when available.
- SAVE Mnemonic for De-Escalation:
 - Support - "Let's work together..."
 - Acknowledge - "I see this has been hard for you..."
 - Validate - "I would probably be reacting the same way if I was in your shoes..."
 - Emotion naming - "You seem upset..."
- Hyperactive Delirium with Severe Agitation refers to a condition where the patient continues to struggle violently despite appropriate restraint that results from a combination of delirium, psychomotor agitation, anxiety, hallucinations, speech disturbances, disorientation, violent and bizarre behavior, insensitivity to pain, elevated body temperature, and uncharacteristic strength. Therefore, underlying etiologies must be considered
- All patients who receive either physical restraint or sedation must be continuously monitored by ALS personnel on scene or immediately upon their arrival. Monitoring must include:
 - Cardiac,
 - pulse oximetry
 - ETCO₂ monitoring

This does not apply if the patient is restrained for law enforcement purposes only and law enforcement is immediately available e.g. the transport of a prisoner in law enforcement custody who is not a behavioral/HDSA patient.

- Any transported patient who is handcuffed or restrained by Law Enforcement should be accompanied by an officer
 - handcuffed patients must never be handcuffed with their arm(s) behind them.
 - Patients may never be handcuffed to the stretcher, other parts of the vehicle, or other pieces of equipment
- Be sure to consider all possible medical and traumatic causes for behavior.
- Restrained patients must never be maintained or transported in a prone position.
- Cold isotonic crystalloid boluses 30 ml/kg with temperature $\geq 104^{\circ}\text{F}$ up to 2 liters in adults.